



Department of Defense Health Agency Performance Work Statement

Clinical Reference Laboratory Testing Services

Date: 19 April 2026

PART 1

1.0 GENERAL INFORMATION: This is a non-personal service(s) contract under which the personnel rendering the service(s) are not subject, either by the contract's terms or by the manner of its administration, to the supervision and control usually prevailing in relationships between the Government and its employees.

1.1 This is a non-personal services contract to provide Clinical Reference Laboratory Testing Services.

1.2 Description of services/introduction: The contractor shall provide all personnel, equipment, supplies, facilities, transportation, tools, materials, supervision, and other items and non-personal services necessary to perform clinical reference laboratory testing services as defined in this Performance Work Statement (PWS) except for those items specified as government furnished property and services. The contractor shall perform to the standards in this PWS.

1.3 Background: Clinical reference laboratory testing includes comprehensive support generally encompassing the American Medical Association's Current Procedural Terminology (CPT) coding within the ranges of 80047-89398, 90281-99607, 99091-99499, 0002M-0020M, 0042T-0900T, and 0001U-0520U. They include, but are not limited to, support across multiple clinical and anatomic pathology disciplines to include chemistry, hematology and coagulation, urinalysis, transfusion medicine, microbiology, molecular, histology, cytology, and genetic testing. Physicians use laboratory results to provide information about an individual's medical condition, and confirmation or exclusion of possible illnesses or disorders. The Government requires performance of commercial reference laboratory testing services for tests currently not supported by in-house DoD testing capabilities or capacity. Requested testing is typically for low volume, high complexity, high cost and/or specialized esoteric testing. Generally, this type of testing has not been brought in-house as it would be cost-prohibitive or inefficient in most cases. Additionally, a significant portion of these are categorized by the United States Food and Drug Administration (FDA) as Laboratory Developed Tests (LDT), i.e., not FDA approved; and are therefore not commercially available outside of the laboratory that developed/validated the assay.

1.4 Objectives: The contractor must be capable of providing comprehensive reference laboratory support which includes all unique, individually orderable tests and panels listed in Exhibit A, Price List and Required Schedule of Tests, across all the aforementioned disciplines. The contractor must possess an infrastructure to fully support the receipt, transport, processing, testing, and reporting of results for the total projected annual requirements across all agencies as defined in PWS Attachment 8 – Performance Requirements Summary. Order and results will be transmitted via the Contractor's Laboratory Information System (LIS) and MHS Genesis.

1.5 Scope: The Contractor shall provide non-personal service(s). Services include clinical reference laboratory testing services to the government at Contiguous/Continental United States (CONUS) and Outside the Contiguous/Continental United States (OCONUS). Clinical reference laboratory testing services will support the Defense Health Agency (DHA) and other Government agencies.

1.5.1 All tests shall be executed at the Contractor's premises, or within the Contractor's network of approved laboratories.

1.5.2 The DHA Contracting Officer's Representative (COR) is the focal point for this requirement. Each submitting activity will have their own Alternate Contracting Officer's Representative (ACOR). In the event of conflicting guidance, the direction provided by the DHA COR shall take precedence.

1.5.3 At a minimum, the contractor will provide specimen processor(s) to DoD sites sending more than 2,000 specimens and expending more than \$30,000 per month for reference lab testing. The current number of sites that fall within these criteria is approximately 16 MTFs, CONUS, and OCONUS, but may increase or decrease over the life of the contract. The current locations that meet this criteria are listed below:

MTF	Location	Average Tests Per Month
Naval Hospital Camp Pendleton	Camp Pendleton, CA 92055 (San Diego)	7,334
Naval Base San Diego	San Diego, CA 92136	18,325
Brian D. Allgood Army Community Hospital	Camp Humphreys, South Korea	2,573
Walter Reed National Military Medical Center	Bethesda, MD 20889	9,668
Brooke Army Medical Center	Ft Sam Houston, TX 78234 (San Antonio)	4,025
Naval Medical Center Portsmouth	Portsmouth, VA 23708	10,024
Tripler Army Medical Center	Tripler AMC, HI 96859 (Honolulu)	5,812
Womack Army Medical Center	Ft Bragg, NC 28310 (Fayetteville)	5,352
Madigan Army Medical Center	Joint Base Lewis-McChord, WA (Lakewood) 98431	2,527
Alexander T. Augusta Military Medical Center	Ft. Belvoir, VA 22060 (Washington D.C.)	5,615
Carl R. Darnall Army Medical Center	Ft. Hood, TX 76544 (Killeen)	7,744
William Beaumont Army Medical Center	Ft. Bliss, TX 79906 (El Paso)	2,859
Landstuhl Regional Medical Center	Landstuhl / Kirchberg, Rhineland-Pfalz, 66849 (Germany)	2,477
Evans Army Community Hospital	Ft. Carson, CO 80913 (Colorado Springs)	3,767
Naval Hospital Jacksonville	Jacksonville, FL 32214	2,455
Naval Hospital Camp Lejeune	Jacksonville, North Carolina 28547	4,937

1.5.4 The contractor will determine a staffing formula for those sites ensuring that specimens are sent out the same day or the next business day. (Required for those sites that send out numerous specimens or requiring manual processing i.e., miscellaneous testing).

1.5.5 The Contractor will have a phase-in period to comply with all requirements and achieve full testing performance. Contractor will provide a phase in plan detailing high-level timelines and major milestones and how requirements will be met, within 14 days of award. This plan will be expanded into a comprehensive Integrated Master Schedule that governs all activities during the six-month transition period defined in PWS 1.6.1. Contractor must be compliant with the current Cyber Security Model Maturity Certification (CMMC) Final Level 2 (Self) as a condition of this contract award.

1.6 **Period of Performance (PoP):** The period of performance for the base single-award indefinite delivery/indefinite quantity contract consists of a five-year base ordering period and a potential five-year optional period for a total of ten years.

1.6.1 Transition: Transition-in/transition-out period. If award is made to a non-incumbent contractor, a transition-in period of up to six-months will be required at the start of the contract. In this scenario, the five-year base ordering period shall commence immediately following the successful completion of the transition. If award is made to the incumbent contractor, no transition-in period will be required. In this scenario, the five-year ordering period shall commence on the contract start date specified in the notice of award.

1.6.1.1 Transition-in period: The Contractor will have up to a six-month transition period to comply with all requirements to accomplish full performance by the start date specified in the Task Order schedule. During the transition-in period, the contractor shall prepare to meet all contract requirements and tasks within this PWS to include:

1.6.1.2 achieve full testing performance, to include completion of their Laboratory Information System (LIS)

and interface

- 1.6.1.3 complete a build of laboratory test file and tables
- 1.6.1.4 complete onboarding and functional training of all qualified specimen processing personnel
- 1.6.1.5 ensure remaining personnel are trained and qualified within 14 days of their start date.
- 1.6.1.6 ensure all cybersecurity requirements are fully adhered to during transition and full performance periods.

1.6.2 Transition-out period

- 1.6.2.1 The contractor shall continue full performance for up to six months
- 1.6.2.2 Cooperate with the incoming contractor to transfer data, processes, and knowledge to facilitate a smooth transition for staff by allowing them to interview with the successor contractor.
- 1.6.2.3 Outgoing contractor will remove contractor items, and clean the area they utilized within the MTF to process specimens being sent to the contractor In Accordance With (IAW) established safety requirements
- 1.6.2.4 Develop a clear plan for managing any samples that are in transit at the time of the handover to ensure they are received and processed correctly by the incoming contractor.

1.7 Administrative specifications

1.7.1 Place of performance: Testing shall be performed at the contractor's facilities and/or contractor's affiliated laboratories.

1.7.2 Recognized Federal holidays: Work shall not be performed by contractor provided specimen processors on U.S federally recognized holidays occurring during the normal workweek unless otherwise directed by the Contracting Officer (CO). When a U.S. holiday occurs on a Saturday or a Sunday, the holiday is observed on the preceding Friday or following Monday, respectively.

New Year's Day	Labor Day
Martin Luther King Jr.'s Birthday	Columbus Day
President's Day	Veteran's Day
Memorial Day	Thanksgiving Day
Juneteenth Day	Christmas Day
Independence Day	

1.7.3 Hours of operation: The contractor is responsible for conducting business 24 hours a day, 7 days a week, except Federal holidays or when the Government facility is closed due to local or national emergencies, administrative closings, or similar Government directed facility closings. The contractor must always maintain an adequate workforce for the uninterrupted performance of all tasks defined in this PWS when the Government facility is not closed. Contractor is required to support an activation or exercise of contingency plans outside the normal duty hours. Support during contingency plans is limited to providing unscheduled STAT courier services and prioritizing the testing of designated specimens IAW the procedures outlined in section 5.3.5.2.

1.7.4 Emergency Services:

1.7.4.1 The contractor program manager shall be available Monday through Friday 0800 through 1700 hrs in their respective region. The contractor representative (CR) shall be able to contact the program manager at any time if an issue needs to be escalated to that level.

1.7.4.2 The contractor shall provide a CR to resolve any technical issues and/or problem areas the customers experience in the field. Refer to paragraph 5.4.3 of this PWS for CR responsibilities.

1.8 Contractor Travel: RESERVED

1.9 Other Direct Costs (ODC): RESERVED

1.10 Quality

1.10.1 Quality Control (QC): The Contractor shall provide a copy of its existing Quality Management Plan (QMP) and QC program to the DHA COR within 15 days after contract award. The contractor shall develop and maintain an effective QC program to ensure services are performed IAW this PWS. The contractor shall develop and implement procedures to identify, prevent, and ensure nonrecurrence of defective services. The contractor's QC program is how the work complies with stated requirements.

1.10.1.1 The contractor shall implement and maintain, throughout the life of the contract, a Quality Management Plan (QMP) and QC program to ensure services are performed IAW this PWS and to ensure compliance with the Clinical Laboratory Improvement Amendment of 1988 (CLIA) and College of American Pathologists (CAP). Current CLIA requirements are located at the Center for Medicare and Medicaid Services website: <https://www.cms.gov/Regulations-and-Guidance/Legislation/CLIA/index.html>.

1.10.1.2 The Contractor's Proposed Quality Control Program (QCP) shall be submitted to DHA through the Contracting Officer's Representative (COR) for approval within 15 days after date of contract award.

1.10.1.3 The government will review and either notify the Contractor in writing of acceptance of the plan or return their comments to the Contractor. If the Government has provided comments, the Contractor shall then have 10 days to submit a Final QCP. After receipt of the Final QCP, the Contractor must receive acceptance by the COR in writing. Any proposed changes to the accepted QCP are required to be resubmitted for acceptance by the CO no later than 15 days prior to the anticipated change and before implementation by the Contractor. The timeline noted above will apply for review and acceptance for proposed changes. At a minimum, the QCP shall address the following:

1.10.1.3.1 A chart showing the organizational structure and lines of authority, the names, contact information, and job title of each member of the Contractor's QC Team.

1.10.1.3.2 How the Contractor will monitor work to ensure performance complies with all deliverables (etc. timelines, deadlines, and goals);

1.10.1.3.3 How the Contractor will monitor work to ensure performance complies with all specifications and requirements of the contract, including the contract's clauses.

1.10.1.3.4 How the Contractor will monitor and ensure staff qualifications remain current and valid including DoD Contractor Personnel Office processes/approvals throughout contract performance.

1.10.1.3.5 How the contractor will inventory and track maintenance of all Government Provided Equipment/Materials.

1.10.1.3.6 How the contractor will identify, investigate, and correct any non-conforming performance and prevent similar deficiencies in the future; and

1.10.1.4 How the contractor shall provide, upon request, to the DHA COR, the following documentation:

1.10.1.4.1 Quality Assurance (QA) policies.

1.10.1.4.2 QC results for assays.

1.10.1.4.3 On-going calculations of QC performance (coefficient of variation, mean and standard deviation).

1.10.1.4.4 Monthly Levy-Jennings graphs.

1.10.1.4.5 Documentation of appropriate analyzer maintenance.

1.10.1.4.6 Verification results for assays of non-regulated analytes.

1.10.1.4.7 Verification results for analytes for which proficiency testing does not currently exist.

1.10.1.5 The contractor shall inform the COR in writing on issues or potential issues that might affect the performance within 12 clock hours of identifying those issues.

1.10.1.6 The contractors shall implement and sustain a QMP and QC Program with records that are complete and accurate throughout the life of the contract.

1.10.1.7 The Contractor shall perform external proficiency testing surveys required by CLIA for all regulated analytes. The Contractor shall provide the DHA COR a listing of external proficiency surveys that the laboratory participates IAW CLIA requirements. Survey results and statistical reviews for specific laboratory specialties shall be provided to DHA COR upon request.

1.10.1.8 Commercial test methods shall be United States FDA approved. LDTs and other non-FDA approved methods require a documented validation plan that complies with all relevant guidelines from CLIA, CAP, the Clinical and Laboratory Standards Institute (CLSI), and the Code of Federal Regulations (CFR). All validation documentation must be made available to the DHA Contracting Officer's Representative (COR) upon request.

1.10.1.9 Upon DHA COR request, the Contractor shall provide CAP interim checklist reports and CAP inspection reports to include deficiencies and corrective actions within 14 days of receiving the report. The Contractor shall submit to the DHA COR notification of completion of corrective action.

1.10.1.10 After acceptance of the QCP the contractor shall receive the COR's acceptance in writing on any proposed change to the QC system

1.10.2 **Quality assurance:** The government will evaluate the contractor's performance under this contract IAW the Quality Assurance Surveillance Plan (QASP). This plan provides a systematic method for the Government to evaluate performance and to ensure that the contractor has performed IAW the performance standards. It defines how the performance standards will be applied, the frequency of surveillance, and the minimum acceptable defect rate(s).

1.11 **Contractor personnel**

1.11.1 **CAC requirements:** A Common Access Card (CAC) is the standard identification for eligible DoD contractor personnel. For all contractors who will work in Government facilities, the Facilities Security Officer (FSO)/Company's Security point of contact (POC) will provide the Government all the required information per the DHA Form 33 Request for Common Access Card, current version (PWS Attachment 5).

1.11.2 **Contractor onboarding and training.** The contractor shall complete all requirements, training, and forms as prescribed in the following requirements: Refer to PWS Attachment 6.

1.11.3 **Physical Security:** The contractor shall be responsible for safeguarding all government equipment, information, and property provided for contractor use.

1.11.4 **Key control:** Reserved.

1.11.5 **Lock combinations:** Reserved.

1.12 **Key personnel (Contractor):** The contractor shall provide a contract manager who shall be responsible for the performance of the work. The name of this person and an alternate who shall act for the contractor when the manager is absent shall be designated in writing to the CO. The contract manager or alternate shall have full authority to act for the contractor on all contract matters relating to daily operation of this contract. The contract manager or alternate shall be available between 8:00 am to 5:00 pm Monday through Friday local time of the primary supported region except Federal holidays or when the government facility is closed for administrative reasons. Qualifications for all key personnel are listed below:

1.12.1 **Contract Manager and Alternate Contract Manager:** The contractor shall provide the CV of the Contract

Manager and Alternate Contract Manager. To minimize Government involvement with contract management, experience with laboratory contracts is preferred but not required.

1.12.2 Quality Manager or Quality Control Manager and Alternate must demonstrate a minimum of the current CLIA qualifications as a General Supervisor within the past ten (10) years, working in Quality Management.

1.12.3 The Contractor shall ensure that all Contractor personnel providing testing services meet the following requirements: The Contractor shall not employ any individual to provide direct health care services who is listed on the Health and Human Services (HHS) list of providers excluded from Federally funded health care programs. The list is found at <http://oig.hhs.gov/fraud/exclusions.asp>.

1.12.3.1 Contractor personnel shall not have been convicted of a felony.

1.12.3.2 Contractor personnel shall successfully complete a criminal history background check IAW paragraph 1.15 of the PWS.

1.12.3.3 Contractor personnel shall be able to read, write, and speak English well enough and enunciate with sufficient clarity to effectively communicate with patients and other providers.

1.12.3.4 CSPs are required to meet Continuing Medical Education requirements as determined by their accrediting organization and/or state.

1.12.3.5 CSP shall have skills, knowledge and experience to perform the specific tasks (see Paragraph 5 of this PWS) listed for their specialty.

1.12.3.6 Contractor and CSPs shall abide by all standards, rules, and procedures including requirements for licensure, credentialing, and QA programs IAW CLIA and credentialing agency.

1.12.3.7 CSP shall be required to take and maintain in a current status, any on-going training as required by regulation, command directive, state law and/or any other inspecting entity.

1.12.3.8 CSPs exhibit competency in the application of microcomputers, data processing operations, relational database software, spreadsheet software, word processing applications, and current operating systems.

1.13 The Government has rights to all deliverables, such as reports and documents, that are created at the Government expense under this contract. For software, this applies only to custom software, scripts, or code newly developed exclusively for the Government under this contract. It does not apply to the contractor's pre-existing or commercially available software, including its proprietary LIS and any modifications or interfaces thereto. All materials supplied to the Government shall be the sole property of the Government with all rights and privileges of ownership and copyright belonging exclusively to the Government and will not be used for any other purpose. These documents and materials will not be used or sold by the Contractor without written permission from the CO.

1.14 **Reporting:**

1.14.1 **Government's COR:** The COR monitors all technical aspects of the contract and assists in contract administration. The COR is authorized to perform the following functions: assure that the contractor performs the technical requirements of the contract; perform inspections necessary in connection with contract performance; maintain written and oral communications with the contractor concerning technical aspects of the contract; issue written interpretations of technical requirements, including Government drawings, designs, specifications; monitor contractor's performance and notifies both the CO and contractor of any deficiencies; and provide site entry of contractor personnel. A letter of designation issued to the COR, a copy of which is sent to the contractor, states the responsibilities and limitations of the COR, especially with regard to changes in cost or price, estimates or changes in delivery dates. The COR is not authorized to change any of the terms and conditions of the resulting contract.

1.14.2 Post award conference/periodic progress meetings: The contractor agrees to attend a post award conference requested by the contracting office. The CO and COR shall meet periodically with the contractor to review the contractor's performance. At these meetings the COR will apprise the contractor of how the government views the contractor's performance and the contractor will apprise the Government of problems, if any, being experienced. Appropriate action shall be taken to resolve outstanding issues.

1.14.2.1 During the Transition In period, the Contractor shall attend a monthly teleconference to discuss and provide a progress report on the 5th of each month with DHA COR. The report and discussion will include progress and issues on the efforts to achieve the required electronic interfaces to ensure secure electronic data transfer capability until it is complete. The Contractor must have the ability to electronically transfer data correctly upon commencement of full performance. Refer to paragraph 1.6.1 of this PWS.

1.15 Contractor Access to Health Affairs (HA)/DHA Network(s):

1.15.1 FSO/Company's Security POC shall notify the DHA Personnel Security Office after being awarded a contract that requires access to a DoD system. Contractor personnel requiring access to the HA/DHA networks for performance of their tasks require a background investigation and the security awareness training. The contractor shall be prepared for this process as it could take two (2) or more weeks. The FSO/Security POC shall submit a Standard Form (SF) 85/86 to DHA's Personnel Security Office for a background investigation. These forms can be found at https://www.opm.gov/forms/pdf_fill/sf85.pdf and https://www.opm.gov/forms/pdf_fill/sf86.pdf respectively.

1.15.2 Company's FSO/Security POC must notify the Personnel Security Office when the contractor has submitted the SF-85/86. The FSO/Security POC must notify the DHA Personnel Security Office in writing of a contractor's termination from the contract, including the termination date.

1.16 Personnel Security:

1.16.1 The contractor shall comply with the following: DoDM "Cyberspace Workforce Qualification and Management Program, dated 15 Feb 2023, DoD Manual (DoDM) 6025.18, "Implementation of the Health Insurance Portability and Accountability Act (HIPAA) Privacy Rule Compliance in DoD Health Care Programs" dated March 3, 2019, Department of Defense Instruction (DoDI) 6025.18 "HIPAA Privacy Rule Compliance in DoD Health Care Programs", dated March 13, 2019; and DoDM 5200.02 "Procedures for the DoD Personnel Security Program (PSP)," incorporation change 3, effective September 24, 2020.

1.16.1.1 Contractor responsibilities for ensuring personnel security include, but are not limited to, meeting the following requirements: Follow the DHA Personnel Security Office guidelines for the submittal of security clearances. Contact the DHA Personnel Security Office for guidance on the appropriate background investigation required for personnel on the contract. The DHA Personnel Security Office can be reached at (703) 275-6038.

1.16.1.2 Initiate, maintain, and document personnel security investigations appropriate to the individual's responsibilities and required access to Controlled Unclassified Information (CUI).

1.16.1.3 DHA Personnel Security Office does not deny any access to any automated information system, network, or Controlled Unclassified Information (CUI). If a contractor receives an unfavorable background investigation, the request for access will be sent back to the FSO for further action. Any unfavorable adjudication will result in DHA Personnel Security Office not signing off on any access request.

PART 2
2.0 DEFINITIONS, ACRONYMS, AND APPLICABLE
PUBLICATIONS/INSTRUCTIONS

2.1 Definitions:

2.1.1 Contracting Officer (CO): A person with the authority to enter into, administer, and/or terminate contracts and make related determinations and findings.

2.1.2 Contracting Officer's Representative (COR): An individual, including a contracting officer's technical representative (COTR), designated and authorized in writing by the CO to perform specific technical or administrative functions. This individual does NOT have authority to change the terms and conditions of the contract.

2.1.3 Nonpersonal services contract: a contract under which the personnel rendering the services are not subject, either by the contract's terms or by the manner of its administration, to the supervision and control usually prevailing in relationships between the Government and its employees.

2.1.4 Quality Assurance Surveillance Plan: An organized written document specifying the surveillance methodology to be used for surveillance of contractor performance. The Government will require the offerors to submit a proposed QASP for the Government's consideration in development of the Government's plan.

2.1.5 Day: means, unless otherwise specified, a calendar day.

2.1.6 Defective Service: means a service output that does not meet the standard of performance associated with the PWS.

2.1.7 Deliverable: means anything that can be physically delivered but includes non-manufactured things such as meeting minutes or reports.

2.1.8 Physical Security: means that part of security concerned with physical measures designed to safeguard personnel; to prevent unauthorized access to equipment, installations, material, and documents; and to safeguard against espionage, sabotage, damage, and theft.

2.1.9 Quality Assurance: (or Government contract QA) means the various functions, including, inspection, performed by the Government to determine whether a Contractor has fulfilled the contract obligations pertaining to quality and quantity.

2.1.10 Quality Control: means all necessary measures taken by the Contractor to assure that the quality of an end product or service shall meet contract requirements.

2.1.11 Statement: means the specific results-based activities required to satisfy High Level Objectives. A statement contains a result, the context of the statement, and the required action(s). Statements focus on "what" is to be accomplished; however, they are not prescriptive in describing "how" the outcome is to be achieved. HLO's have several statements to flesh out the areas necessary to meet the objective. Statements are similar to Level 3 in a Work Breakdown Structure.

2.1.12 Subcontractor: means one that enters into a contract with a prime Contractor. The Government does not have privity of contract with the subcontractor.

- 2.1.13 **Work Day:** The number of hours per day the Contractor provides services IAW the contract.
- 2.1.14 **Work Week:** Monday through Friday, unless otherwise specified.
- 2.1.15 **Accreditation:** The formal approval by the DHA to operate the contractor's IS/networks in a particular security mode using a prescribed set of safeguards at an acceptable level of risk. In addition, accreditation allows IS/networks to operate within the given operational environment with stated interconnections, and with appropriate level of protection for the specified period.
- 2.1.16 **Certification:** The determination of the appropriate level of protection required for IS/networks. Certification also includes a comprehensive evaluation of the technical and non-technical security features and counter measures required for each system/network.
- 2.1.17 **Contract Service Personnel/Provider (CSP):** An employee of the Contractor, subcontractor and/or subcontractor personnel or providers. (i.e., Contractor representative, customer service personnel, data entry/specimen processing technician, and courier)
- 2.1.18 **Current Procedure Terminology (CPT):** A listing of descriptive terms and identifying codes for reporting medical services and procedures performed by physicians.
- 2.1.19 **Etiological Agent:** Infectious agents as defined by 42 CFR Part 72.
- 2.1.20 **Laboratory Developed Test (LDT):** An in vitro diagnostic test that is manufactured by and used within a single laboratory.
- 2.1.21 **Logical Observation Identifier Names and Codes (LOINC):** The LOINC Laboratory terms set provides a standard set of universal names and codes for identifying individual laboratory and clinical results. LOINC Codes allow users to merge clinical results from many sources into one database for patient care, clinical research, or management.
- 2.1.22 **Medical Expense and Reporting System (MEPRS) Code:** An ordering code (three to four alpha-numeric characters) that identify the sub-specialty within the hospital to charge expenses. As an example, code AAAA, is charged to Inpatient Internal Medicine Services.
- 2.1.23 **MHS Genesis:** The electronic health record (EHR) for the Military Health System (MHS), provides enhanced, secure technology to manage your health information. MHS GENESIS integrates inpatient and outpatient solutions that will connect medical and dental information across the continuum of care, from point of injury to the MTF. This includes garrison, operational, and enroute care, increasing efficiencies for beneficiaries and healthcare professionals.
- 2.1.24 **Medico-Legal:** of or relating to both medicine and law.
- 2.1.25 **Quality Assurance Program (QAP):** All necessary measures taken to assure that the quality of an end product or service shall meet contract requirements.
- 2.1.26 **Shipping Form:** Form(s) used to initiate shipping actions.
- 2.1.27 **Specimen:** A quantity of human blood, serum, tissue, excreta, patient body fluid, or environmental specimen provided for test purposes.

2.1.28 STAT: Immediately

2.1.29 Submitting Activity (SA): Any Government activity submitting specimens under this contract.

2.1.30 Submitting Activity Contracting Officer (SA CO): Contracting Officer who issues an individual task order against the ID/IQ contract, other than the CHCC/CO

2.1.31 Transfer Tube: Any tube used to collect, transport, transship, and store samples under terms of the PWS.

2.1.32 Trusted Associate Sponsorship System: Web-based system which replaces the Contractor Verification System (CVS)

2.1.33 Turn Around Time: The time from when a testing facility receives a specimen to the time the specimen is resulted in the IS.

2.2 **Acronyms:**

B2B	Business-2-Business
CAC	Common Access Card
CAP	College of American Pathologists
CDRL	Contract Data Requirement List
CFR	Code of Federal Regulations
CLIA	Clinical Laboratory Improvement Amendments of 1988
CLMS	Center for Laboratory Medicine Services
CLSI	Clinical Laboratory Standards Institute
CMMC	Cybersecurity Maturity Model Certification
CNSSI	Committee on National Security Systems Instruction
CO	Contracting Officer(s)
CONUS	Continental United States (excludes Alaska and Hawaii)
COR	Contracting Officer Representative
CPT	Current Procedure Terminology
CSP	Contract Service Personnel
CUI	Controlled Unclassified Information
DD Form 254	Department of Defense Contract Security Requirement List (if applicable)
DFARS	Defense Federal Acquisition Regulation Supplement
DHA	Defense Health Agency
DoD	Department of Defense
DoDD	Department of Defense Directive
DoDI	Department of Defense Instruction
DSAA	Data Sharing Agreement Application
DMDC	Defense Manpower Data Center
DoDM	Department of Defense Manual
DPCLO	DHA Privacy and Civil Liberties Office
eMSM	Enhanced Multi-Service Markets
FDA	[United States] Food and Drug Administration
FRCS	Facility Related Control Systems
FSO	Facilities Security Officer
GFP	Government Furnished Property
HIPAA	Health Insurance Portability and Accountability Act
HHS	Health and Human Services
IA	Information Assurance

IATA	International Air Transport Association
IAW	In Accordance With
ID	Identification
IT	Information Technology
LDT	Laboratory Developed Test
LIS	Laboratory Information System
LOINC	Logical Observation Identifier Names and Codes
MHS	Military Health System
MTFs	Military Treatment Facilities
NIST	National Institute of Standards and Technology
OCNUS	Outside Continental United States (includes Alaska and Hawaii)
ODC	Other Direct Costs
OPM	Office of Personnel Management
OPSEC	Operations Security
PHI	Protected Health Information
PII	Personally Identifiable Information
POC	Point of Contact
PRS	Performance Requirements Summary
PWS	Performance Work Statement
QA	Quality Assurance
QASP	Quality Assurance Surveillance Plan
QC	Quality Control
QCP	Quality Control Plan
SF	Standard Form
SPRS	Supplier Performance Risk System
MEPRS	Medical Expense and Reporting System
NACLC	National Agency Check with Law Enforcement and Credit
SA	Submitting Activity
SA COR	Submitting Activity Contracting Officer Representative
SHARP	Sexual Harassment/Assault Response and Prevention
TARP	Threat Awareness & Response Program
TAT	Turn Around Time

2.3 Applicable Publications, DHA Administrative Instructions (AI), etc.

- 2.3.1 Department Of Defense Contract Security Classification Specification (DD Form 254)
- 2.3.2 Department of Defense Security Agreement (DD Form 441)
- 2.3.3 National Industrial Security Program Operating Manual (DoD 5220.22-M)
- 2.3.4 Information Assurance Workforce Improvement Program (DoD 8570-M)
- 2.3.5 Information Assurance Training Certification and Workforce Management Directive (DoDD 8570.01)
- 2.3.6 Committee on National Security Systems (CNSS) Glossary”, 6 April 2015
- 2.3.7 CNSSI No. 1253 “Committee on National Security Systems (CNSS) Instruction No. 1253, Security Categorization and Control Selection for National Security Systems, 27 March 2014
- 2.3.8 DoD Manual 5200.2, DoD Personnel Security Program (PSP)
- 2.3.9 DoD 6025.18-R, “DoD Health Information Privacy Regulation”
- 2.3.10 DoD Instruction 8520.02, Public Key Infrastructure and Public Key Enabling\DoD

Instruction 8551.1, Ports, Protocols, and Services Management

- 2.3.11 DoD Instruction 8580.02, Security of Individually Identifiable Health Information in DoD Health Care Programs
- 2.3.12 DoD Instruction 6025.18, Privacy of Individually Identifiable Health Information in DoD Health Care Programs
- 2.3.13 DoD Directive 5400.11, DoD Privacy Program
- 2.3.14 DHA Memorandum “Update-Risk Management Framework for Medical Community of Interest Migration”, 2 May 2017
- 2.3.15 DHA Administrative Instruction 77, “Security Categorization (SC) and Control Selection for Information Technology (IT)”, 28 May 2015
- 2.3.16 Federal Information Processing Standard (FIPS) Publication 199, “Standards for Security Categorization of Federal Information and Information Systems”, February 2004
- 2.3.17 FIPS Publication 200, “Minimum Security Requirements for Federal Information and Information Systems”, March 2006
- 2.3.18 FIPS Publication (FIPS PUB) 140-2, Security Requirements for Cryptographic Modules
- 2.3.19 FIPS PUB 201-2, Personal Identity Verification of Federal Employees and Contractors
- 2.3.20 NIST SP 800-30, “Guide for Conducting Risk Assessments”, Revision 1. September 2012
- 2.3.21 NIST SP 800-37 Revision 1, “Guide for Applying the Risk Management Framework to Federal Information Systems: A Security Life Cycle Approach”, February 2010
- 2.3.22 NIST SP 800-39, “Managing Information Security Risk: Organization, Mission, and Information System View”, March 2011
- 2.3.23 NIST SP 800-53 “Security and Privacy Controls for Federal Information Systems and Organizations”, Rev 4, April 2013
- 2.3.24 NIST SP 800-53A Revision 4, “Assessing Security and Privacy Controls in Federal Information Systems and Organizations”, December 2014
- 2.3.25 NIST SP 800-60 Volume 1 Revision 1, “Guide for Mapping Types of Information and Information Systems to Security Categories”, August 2008
- 2.3.26 DODI 1400.25, “Civilian Personnel Management”
- 2.3.27 DODI 2000.16 “DoD Antiterrorism Program”, 8 May 2017
- 2.3.28 DODI 8582, “Security of Non-DoD Information Systems Processing Unclassified
- 2.3.29 Nonpublic DoD Information”, 09 December 2019.
- 2.3.30 Local regulations as deemed appropriate by the Information Assurance (IA) Officer specified at the task order level.

PART 3

3.0 GOVERNMENT FURNISHED PROPERTY, EQUIPMENT, AND SERVICES

The Requiring Activity Authority has assessed the need for Government Furnished Property, Equipment, and Services and determined:

3.1 Services: The Government: Provide biologic specimens that have been collected and processed (centrifuged, separated, labeled, and preserved) per the Contractor's published specimen collection requirements, and IAW standard quality procedures.

☒ **WILL** provide Government Furnished Services required in support of this contract/task orders these services are described below:

3.1.1 Perform specimen collection.

3.1.1.1 Order required tests in MHS Genesis or other LIS, when available, and provide the Contractor with test requests, lists, or work documents with required patient demographics and ordering information. Once the lab test file is interfaced, the Government will not complete additional Contractor forms, data entry, or test requisitions in the Contractor's automated laboratory system. For each test requisition, Government personnel will provide:

- Patient's full name.
- Department of Defense Identification (DoDID) Number or Unique Patient Identifier.
- Submitting Activity's Name and Accession Number.
- Date of Specimen Collection.
- Date Specimen Shipped.
- Test(s) Requested/Code and Name.
- Specimen Type.
- Ordering Physician's Name.
- Ordering Remarks/Comments (if clinically appropriate).
- Patient's Age or Date of Birth.
- Patient's Sex.
- Submitting Activity Site.
- Submitting Activity Site Account Number.

3.1.1.2 Specimens for testing

3.1.1.3 If onsite CSPs are provided, The Government will provide workspace sufficient to accommodate the required on-site contractor personnel, including desk space, storage for supplies, and access to required utilities.

3.2 Facilities: The Government

☒ **WILL** provide Facilities in support of this contract/task orders. The Government provided Facilities are described below:

3.2.1 The Government will provide sufficient workspace, to include desk space, storage space, for CSPs to conduct their work as outlined in the PWS.

Utilities: The Government:

☒ **WILL** provide Utilities in support of this contract/task orders. The Government provided Utilities are described

below:

3.2.2 Government will provide all utilities necessary for CSPs to conduct their work IAW PWS. The Contractor shall instruct employees in utilities conservation practices. The Contractor shall operate under conditions that preclude the waste of utilities, which include turning off the water faucets or valves, light switches, etc. after using the required amount to accomplish tasks requiring the use of the utilities.

3.3 **Equipment:** The Government:

☒ **WILL** provide Equipment in support of this contract/task orders. The Government will provide equipment necessary for CSPs to perform their duties IAW PWS.

3.4 **Procurement Integrated Enterprise (PIEE), GFP Module Application:** Reserved.

3.5 **Materials:** The Government:

☒ **IS** providing Materials in support of this contract/task orders. The Government-provided Materials are described below:

3.5.1 The Government will provide all pertinent SOPs, policies, and procedures necessary for CSPs to perform work in the facility, ensuring compliance with facility directives.

PART 4
4.0 CONTRACTOR FURNISHED ITEMS AND SERVICES

4.1 **Services:** The Contractor:

☒ **WILL** provide Contractor Furnished Services required in support of this contract/task orders. These Services are described below:

4.2 **General:** The contractor shall furnish all supplies, equipment, facilities and services required to perform work listed under Part 5 of this PWS.

4.3 **Equipment:** The contractor shall provide their own vehicles and equipment to perform the work described in this PWS. Contractor's vehicles and equipment used in the performance of this contract shall meet all local and Host Nation traffic, safety, and environmental requirements.

4.4 **Facilities:** Reserved

PART 5

5.0 SPECIFIC TASKS

5.1. Contractor Furnished Property and Services

5.1.1. The contractor shall be responsible for completing the following:

5.1.1.1. General:

- Test specific specimen collection and preservation requirements for each test or an analyte.
- Test specific method and interpretation for each test or analyte.
- Test specific method reference ranges, adjusted for age, sex, or race, when applicable.
- Test specific method sensitivity, specificity, and interferences, when required.
- Test specific schedule of performance.
- Average turnaround time for each.
- Test Order Code
- CPT Code(s)
- Result Code
- Testing Location

5.1.1.1.1. Updated Information on Test Changes (i.e. changes in normal ranges, methods, and reporting units, etc.). In the event of any methodology change, the contractor must have an effective notification system to provide the DHA COR, SA COs, and CO for each task order and each SA COR within 15 calendar days prior to the Contractor implementing said change. This will allow time for updating MHS Genesis test files and informing Medical Staff.

5.1.1.1.2. Contract Evolution and Technology Insertion: The services required under this contract are expected to evolve with advancements in medical technology and diagnostic practices. Accordingly, this contract contemplates the on-going insertion of new technology and the refreshment of existing equipment and services. The Government, at its sole discretion, may incorporate such advancements at any time during the period of performance in accordance with the terms and conditions of this contract. The mechanisms for proposing, evaluating, and integrating these advancements are defined in the Special Contract Requirements Section of this resulting contract.

5.1.1.1.3. Product Information. The Contractor shall provide product information on new test(s) offered by the Contractor to the DHA COR.

5.2. **Basic Services:** The Contractor is providing necessary services required to perform clinical reference laboratory testing services including, supplies, labor, customer services, courier services, software, equipment, computers, IT, reporting/interpreting, result reporting, and management reports (refer to 5.3.16.3 – 5.3.16.9). The Contractor shall provide electronic interfaces between the Contractor's LIS and MHS Genesis for order entry and result retrieval. In emergency / contingency situations the Contractor shall use a secure web-based system. In case the web-based system is inoperable, courier services will be used for order entry and result retrieval. These services will be provided to Military Treatment Facilities (MTFs) via task order issued by each ordering activity. The Contractor shall comply with all provisions of the Privacy Act of 1974 (Public Law 93-579) and HIPAA when providing Reference Laboratory Services.

5.2.1. The Contractor shall ensure accurate and timely performance of laboratory testing services in biologic specimens provided by Submitting Activities (SAs). The services provided must comply with and conform to all best practices as outlined by CLIA regulations, CAP standards, and all applicable state laws and regulations, and

where applicable, New York State Department of Health licenses. Prime vendor (primary contract holder) must have and maintain accreditation with the College of American Pathologists (CAP). Subcontractors utilized by the prime vendor to perform clinical laboratory testing must have and maintain accreditation with a Health and Human Services (HHS) approved laboratory accreditation agency.

5.2.2. Laboratory directors shall be American Board of Pathology certified.

5.3. Tasks: Tasks consist of the following:

5.3.1. **TASK - Materials:** The contractor shall provide initial stock upon award. The Contractor shall provide an initial 8-week stock of materials, supplies and testing kits, to be used by the SAs identified in each task order, in preparation for the collection and shipment of specimens. The Contractor shall meet with SA COR during the transition phase to determine the quantity required for each site.

5.3.1.1. Sustainment/Ongoing stock:

5.3.1.1.1. The Contractor shall be responsible for providing test-specific collection kits and supplies needed for the shipment of specimens from SA to Contractor. Shipping and packaging containers and materials shall be capable of maintaining temperature requirements for specimens until they reach the Contractor's laboratory under the terms of this contract.

5.3.1.1.2. Required labels and packaging materials for shipping specimens via courier that are infectious, or etiologic agents, are IAW appropriate requirements of 42 CFR Part 72, 49 CFR Parts 171 and 173, and the Dangerous Goods Regulations of the International Air Transport Association (IATA) consistent with current regulatory updates.

5.3.1.1.3. Test request forms, preprinted with the appropriate Shipping Section and account information for the tests offered. Test request forms for specialized testing (i.e., cytogenetics, tissue, etc.). Forms and special handling procedures for medico-legal specimens when required by the Government.

5.3.1.1.4. Impervious bags for specimen pick-up. Bags shall have at least two sections, one for specimen(s) and one for paperwork.

5.3.1.1.5. Printer supplies including paper and labels, laser printer consumable supplies such as toner and cartridges in sufficient quantities to perform work under this contract.

5.3.1.1.6. Barcode labels for identification on each sample transfer tube, or specimen container if requested. (Labels furnished by printing with the Contractor-provided LIS).

5.3.1.1.7. Written instructions to Government personnel for sample preparation, applying barcode labels, packing samples, preparing paperwork, and shipping or transport of containers.

5.3.1.1.8. The Contractor shall furnish materials, supplies, equipment, and reagents necessary to meet the requirements for the provision of services established under the terms of this contract.

5.3.1.1.9. The Contractor shall provide maintenance and replacement of furnished items to the Government for the entire contract period. The Contractor shall repair and/or replace equipment within 24 hours of notification by the Government of equipment failures.

5.3.1.1.10. The Contractor shall provide a 24-hour toll-free customer service/Help Desk telecommunication lines (including installation and maintenance service charges) to support incoming Government personnel calls for assistance.

5.3.1.1.11. Submitting Activities with on-site contracted Specimen Processor(s) will maintain stock of required supplies needed to collect and ship specimens. Maintenance of the stock is the responsibility of the Contractor.

5.3.1.1.12. Submitting Activities without an on-site contracted Specimen Processor(s) will be provided ordering instructions on how to request test-specific collection kits and supplies needed for the shipment of specimens from SA to Contractor.

5.3.1.1.13. The cost of the kits and shipping and collection supplies are the responsibility of the Contractor.

5.3.1.1.14. Supplies shall be replenished by the Contractor to allow for performance of all contract requirements without interruption.

5.3.1.1.15. On a case-by-case basis, emergency supplies will be requested by each SA POC and shall be provided by the Contractor. Emergency supply requests will be shipped via overnight delivery at the contractor's expense for receipt on the next business day.

5.3.2. **Task - Testing.** The Contractor shall provide the full range of clinical reference laboratory testing capabilities to execute required tests defined in the Contractor's Price List/Required Schedule of Tests (Exhibit A). All tests shall be uniquely identified with a single test number that is standardized throughout the Contractor's network of laboratories. All clinical reference laboratory testing, and anatomic pathology specimens, shall be executed IAW CLIA regulations, CAP, and all applicable state laws and regulations.

5.3.2.1. The Contractor shall not charge any set-up fees for cytogenetic tests showing no growth.

5.3.2.2. If a medico-legal specimen is submitted, the Contractor shall provide its own special forms and special handling procedures to maintain valid "chain-of-custody possession" and develop the formal documentation necessary for that purpose. The Contractor's testing personnel that performed the analysis can be required to provide Court testimony. Contractor testimony shall be provided as required to the Government. The costs associated with providing medico-legal testimony, including travel and labor hours, are not included in the fixed price of this contract. Such services, when required by the Government, will be considered an Other Direct Cost (ODC) and will be billed to the Government at pre-negotiated labor rates with prior approval from the Contracting Officer. If a specimen is required for medico-legal issues, at the request of the Government, the specimen shall be retained indefinitely.

5.3.2.3. The Contractor shall provide a mechanism for large batch specimen ordering, processing, and transport.

5.3.3. **Task – Labor.** The Contractor shall provide labor to:

5.3.3.1. Process and ship specimens to and from Contractor and Government sites.

5.3.3.2. Provide specimen processing personnel to government sites processing more than 2,000 specimens/month with expenditures exceeding \$30,000.

5.3.3.2.1. Center for Laboratory Medicine Services (CLMS) reserves the right to address assignment of processors with the vendor and will evaluate assignment of processes based upon the annotated variables on a quarterly to semiannual bases.

5.3.3.3. Determine a staffing formula for those sites assigned processors to ensure that specimens are sent out the same day or no less than the following day. (Required for those sites that send out numerous specimens or requiring manual processing i.e., miscellaneous testing).

- 5.3.3.4. Process and ship specimens to and from Contractor and Government sites.
- 5.3.3.5. Order and ship supplies.
- 5.3.3.6. Enter laboratory results into MHS Genesis as necessary.
- 5.3.3.7. Enter laboratory test requests, retrieve laboratory results, and obtain testing information.
- 5.3.3.8. Print utilization reports to and from Contractor and Government sites.
- 5.3.3.9. Provide 24-hour customer service functions to include assisting with computer help desk requests, such as responding to field queries, requests to track testing progress and test reporting, responding to requests for assistance to correct/answer on-site computer system and Government remote site equipment problems, questions, and failures to operate.

5.3.4. **Task – Customer Service.** The Contractor shall provide a toll-free number for customer services. The services shall be available 24 hours per day, 7 days per week. Through this service Government staff at each SA shall be assisted with the following: tracking test results, modifying test requests, obtaining information regarding specimen requirements, requesting emergency courier service, technical support for use of Contractor's computer system, and other applicable related issues/problems that arise in the performance of work under this contract.

5.3.4.1. The Contractor shall ensure that Government personnel receive quality customer service through the assistance of knowledgeable and responsive laboratory professionals. Transfers will be kept at a minimum and hold times will be IAW the standards set in the contractor's proposal.

5.3.5. **Task – Courier Services.** The Contractor shall pick up, ship, and deliver specimens from the SA site to the Contractor laboratory. The Contractor shall maintain proper tracking of the specimens from the initial pick-up of specimens from SA site throughout the testing process at the Contractor's reference laboratory.

5.3.5.1. Routine Clinical Reference Laboratory Testing Services. The Contractor shall provide routine local courier pick-up services at each SA daily (Monday – Friday, excluding federal holidays) at a time mutually agreed upon by the SA COR and the Contractor.

5.3.5.2. Unscheduled/STAT Clinical Reference Laboratory Testing Services. The Contractor shall accommodate additional specimen pick-ups for unscheduled testing, 24 hours a day, 7 days a week, for each SA located in both the CONUS and OCONUS. Courier pick up time must be within 3 hours after notification by any SA. Costs for unscheduled specimen transport are not included in routine courier fees and will be charged separately in accordance with the rates specified in Exhibit A, Price List.

5.3.5.3. If the electronic reporting requirements of this contract are temporarily unavailable for use, the Contractor shall provide expedited overnight courier shipping, mailing, and/or transportation services to pick-up specimens for testing or return of printed reports of completed test results.

5.3.5.4. The courier will transport specimens under required conditions to ensure viability and stability for testing.

5.3.6. **Task – Catalog of Laboratory Services.** The Contractor shall provide a catalog and an electronic version/website of the catalog shall be made available to government personnel at the medical treatment facilities and SA COR. The catalog will be used by the government as a reference to determine the specimen requirements, methodology and clinical utility of the tests offered by the contractor.

5.3.7. **Task – Laboratory Information System Interface.** The Contractor shall provide an LIS bidirectional interface with MHS Genesis (or current Government LIS).

5.3.7.1. In cases of malfunction of the Contractor LIS Interface lasting more than 48 hours, the Contractor shall, without additional charge, provide express overnight shipping, courier delivery, or fax to each SA until the LIS Interface is properly functioning. The method will be mutually agreed upon by the SA COR.

5.3.7.2. Contractor Laboratory Information System. The Contractor shall have the ability of authorized individuals at the SA to access a web-based system to manually build a patient information record, enter test requests, view results, print patient reports, access patient test history, and view specimen collection requirements during the term of the contract.

5.3.8. **Task - Test Results.** Test result information shall also be available on-line through "patient information" records. Reprints of patient test results using the system shall be available for at least 2 years or to meet the CAP guidelines.

5.3.8.1. The web-based system shall have the following or equivalent menu selections:

- Browse by shipping facility accession number.
- Browse by Contractor's identification.
- Browse result by patient name.
- Browse result by patient identifier (e.g., DOD Identification (ID) Number).
- List requests with no test results (Pending Results).
- List overdue results.
- Change entry password.
- Ability for convert test information to an excel spreadsheet for export.

5.3.8.1.1. Online Test Menu Catalog. The test menu catalog will be used by the government as a reference to determine the specimen requirements, methodology, and clinical utility of all the tests offered by the contractor. On-line access to detailed reference data, such as might be normally available through a hardcopy. Such data shall include all items listed in paragraph 5.1. and must include updates as described in paragraph 5.1.1.1.1.

5.3.8.1.2. Patient Information Backup Capability. The information system (IS) shall have the ability to develop and maintain backup patient information and shall be available daily, including test results processed.

5.3.9. **Task – Equipment.** The Contractor shall furnish and install all necessary equipment needed to perform electronic data entry, provide test/assay information complete testing, store and print transmitted results, transfer data, view digital images associated with results, manage operations, label and identify specimens, print chartable patient report, and upload test results into MHS Genesis or current Government LIS for the life of the contract. Training shall be provided at each SA as needed and necessary.

5.3.10. **Task – Results Reporting.** Routine test results shall be electronically transmitted through the interface, to each SA, at a minimum, every morning and afternoon (except Sunday) by 7:00 a.m. and 2:00 p.m. (local time), respectively.

5.3.10.1. The Contractor shall provide results of laboratory testing to the SA within the Contractor's established turnaround time IAW its procedures manual and Exhibit A, Schedule of Required Tests. Contractor's turnaround times will be incorporated into the resultant contract.

5.3.10.2. Individual patient-chartable reports meeting CLIA requirements shall be electronically transmitted to each SA for local printing in the Shipping/Receiving Section of each SA. As a minimum, printed reports shall include:

- The name and address of the contract laboratory where each test was performed.
- The specimen accession number of the contract laboratory.
- The name of the contract Laboratory Director or Medical Director.
- A phone number to contact the contract laboratory concerning the test results.
- The contract laboratory's CLIA certification number.
- The patient's name.
- The patient's DoD Identification Number or Unique Patient Identifier.
- The patient's age or date of birth.
- The patient's sex.
- The SA specimen identification number.
- The requester's MEPRS code.
- The specimen collection date.
- The Contractor's report date and time.
- The test name.
- The test result(s).
- The unit of measurement for quantitative tests.
- The normal or expected range (reference range adjusted for age, sex, or race when appropriate).
- The report status (i.e., final or interim).
- Other interpretive information appropriate for the test(s).

5.3.10.3. Abnormal results (results outside the reference range) shall be flagged by some indication on the reports to differentiate them from normal results (results within the reference range).

5.3.10.4. The Contractor shall provide genetics consultation to healthcare providers at no additional cost as needed for interpretation of genetic testing and results.

5.3.10.5. The Contractor shall provide names and telephone numbers of technical directors and pathologists available for consultation.

5.3.11. **Task – Reporting and Interpretation of Critical Results and Amended Results.** Critical results shall be telephoned, within 30 minutes, directly to the SA after being confirmed and certified by the Contractor. Electronic transmission with be IAW Contractor's stated procedure.

5.3.11.1. Amended results shall be telephoned directly to the SA after being confirmed and certified within 30 min by the Contractor, IAW Contractor's stated procedure. The telephonic report shall be followed by an electronic transmission within one (1) day. Amended results must also be flagged and copies of revised reports must be provided to the SAs. The report shall be an individualized chartable patient report.

5.3.11.2. Non-conforming events, or erroneous testing results, caused by laboratory testing error(s) (e.g. equipment failure, undetected QC failure, specimen processing error, etc.) must be reported directly to the DHA COR within 24 hours of discovery. Following the initial verbal or electronic notification to the DHA COR, a full report detailing the incident, investigation, corrective actions with monitoring, and a summary of all amended patient reports must be submitted to the DHA COR within 30 days.

5.3.12. **Task – Reporting STAT Requests.** The Contractor shall submit the procedure(s) for testing and reporting STAT test results to each SA upon award of each task order.

5.3.13. **Task – Handling of Specimen or Request Discrepancies.**

5.3.13.1. Specimen loss due to breakage, spillage/leakage, insufficient dry ice, misplacement, mishandling or

any other Contractor-related problem(s) shall be reported immediately to the impacted SA upon knowledge or detection of problem. Specimen loss, calculated on a per-task-order basis and measured monthly, shall not exceed 1% of the total specimens submitted by that task order's Submitting Activities within that month. Repeated problems with compromised specimens shall be grounds for termination for cause of the contract.

5.3.13.2. Any specimens that have been broken, leaked, or otherwise compromised during transit shall be immediately reported to the appropriate SA upon unpacking at the Contractor's laboratory. The Contractor is required to pick-up the new specimen at no additional cost to the Government. The pick-up of the new specimen shall be made during the routine courier pick-up times; if emergency pick-up is required due to contractor error, then the unscheduled/Stat fee schedule does not apply.

5.3.13.3. All specimens shall be processed and shipped within one business day of receipt. Specimen discrepancies or additional specimen requirements shall be brought to the attention of the SA within one business day after knowledge of discrepancy.

5.3.13.4. The Contractor shall provide specimen samples to each SA upon request for correlation and validation studies as needed at no additional cost. Specimens will be sent to the SA within 72 hours of the request.

5.3.14. **Task – Specimen Retention.**

5.3.14.1. Specimens shall be stored and retained by the Contractor for at least seven (7) days after the reporting of the results. In selected cases, and upon request by a site, the Contractor shall store/retain specimens for up to 30 days at no additional cost. If necessary, the SA will request repeat determinations or add-on tests to stored specimens, by telephone during this 7-day or extended 30-day period. Telephonic request orders made by the SA must be placed by the Contractor into their LIS. Repeat determinations of the original test, requested due to clinically inconsistent results, shall be performed at no cost to the Government. New 'add-on' tests ordered on stored specimens will be considered a new order and will be billed at the contracted price for that test. Specimen retention applies to blood, serum, Cerebral Spinal Fluid, urine, and other body fluids.

5.3.14.2. All anatomic pathology specimen(s) (e.g., blocks, slides or other diagnostic material) shall be returned by courier or expedited shipping methods to the submitting facility within seven (7) calendar days after final diagnosis. The cost of returning all anatomic pathology specimens is included in the price of the associated test.

5.3.15. **Task – Referrals to Another Laboratory or Institution.** The contractor shall ship specimens to other laboratories for tests on the Contractor's Price list that are not performed by the Contractor's network of laboratories. The Contractor shall ship specimens to other laboratories for tests that are to be added to the pricelist by modification as described in paragraph 5.3.16.13.

5.3.16. **Task - Contractor Project Management.** Contractor shall manage all assigned projects from initial receipt until project completion. The Contractor shall ensure all projects are completed IAW the Contractor's approved schedule and project cost as noted below.

5.3.16.1. The Contractor must attend a monthly teleconference to discuss and provide a progress report on/around the 5th of each month between the DHA and the Contractor. The report and discussion will include progress and issues on the efforts to achieve the required electronic interface to ensure secure electronic data transfer capability, until it is complete.

5.3.16.2. The contractor shall submit the procedures for notifying appropriate Government personnel of critical value results, STAT test results, IAW regulatory requirements to the SA upon award of each task order.

5.3.16.3. Monthly SA Reports. The Contractor shall provide separate monthly data summary reports for test performed and test not performed for each SA under this contract. The printed reports shall be delivered to each SA CO and to each SA COR as identified on each task order, by the tenth calendar day of the month after the end of each monthly reporting period. Contractor shall provide reports to the Regional Lab Managers, upon request, for each SA in that region.

5.3.16.4. Monthly SA reports shall include the following for each test performed:

- Test Name
- CPT Code(s)
- Vendor Test Code (if different from CPT code)
- FDA Approval status of test.
- Test methodology
- Cost of test (per test)
- Subtotal of test performed current month.
- Subtotal of cost, current month.
- Subtotal of test performed, year to date.
- Subtotal of cost of test, year to date.
- Performing lab site name.
- Changes to the tests name, CPT code, vendor's test code and revised cost per test shall have the effective date annotated in the report.

5.3.16.5. For test(s) not performed (i.e., rejected samples) reported information shall include subtotal by reason/rejection category (e.g., Quantity Not sufficient) and include the following:

- Patient name
- Patient ID
- Specimen ID number
- Vendor test name
- Vendor test code
- Rejection code and associated descriptions
- Any additional lab entered comments associated with the rejected sample
- Performing laboratory
- Date of rejection

5.3.16.6. The reports with data required in paragraph 5.3.16.4 shall be sorted by three different methods:

- Test names listed alphabetically.
- Test names listed in order of year-to-date test frequency (highest to lowest).
- Test names listed in order of year-to-date total test cost (highest to lowest).

5.3.16.7. Monthly Service Agency Reports . The Contractor shall provide monthly data summary reports of tests performed and not performed for each SA within the respective Service Agencies/DoD Branch to the DHA COR, CO, and Service Agency Leads. This report will include the information listed in paragraph 5.3.16.4 with the following additional fields:

- DHA MTFs Submitting Activity MTF Type.
- The reports with data required in paragraph 5.3.16.5 shall be sorted by Service Agency and the same three methods outlined in paragraph 5.3.16.6.
- DHA Networks (Upon request).

- The reports with data required in the PWS paragraph 5.3.16.4 shall be sorted by DHAs Network using the same three methods outlined in paragraph 5.3.16.6.

5.3.16.8. Critical Results Report. The contractor will provide the COR a monthly summary report of the number of critical results. This report will include:

- SA.
- Test.
- Value.
- Normal Range.
- Time and Date test was certified.
- Time result was called to the SA.
- Name of individual accepting the results.

5.3.16.9. Quarterly TAT Report. The Contractor shall provide Turnaround Time (TAT) reports of laboratory testing to each SA. The TAT report shall compare actual TAT with established TAT IAW its procedure manual. The TAT standards shall be met at least 98% of the time.

5.3.16.10. All reports shall be submitted in electronic spreadsheet form. Electronic supplemental narrative shall be forwarded as appropriate.

5.3.16.11. The Contractor shall provide operational and managerial reports as required.

Minor modifications to existing reports will be made at no additional cost. Requests for entirely new reports or significant modifications requiring substantial development effort will be negotiated and implemented via a contract modification.

5.3.16.12. Notification of when a specific test is subject to a unit price reduction based on the Contractor's proposed discount schedule/terms incorporated under contract shall be provided by the Contractor to the DHA COR, and the CO. Identify the test name, CPT code, vendor's test code (if different from CPT code), and revised unit cost per test.

5.3.16.13. Notification of the revised pricing for a given test based on price reductions will be made to the DHA COR and notification provided to SA CORs and other COs by modification to the basic contract. Change will be reflected in the Contractor's price list as well.

5.3.16.14. The contractor shall designate and submit, in writing, within 10 working days after award, the name, telephone number, pager, cell phone number, facsimile number and e-mail address of a full-time Contractor representative and alternate. The written designation shall be submitted to the DHA COR, CO, and SA COR as identified on contract or task order.

5.3.17 **Task – Mission Essential Contractor Services Plan.** These services have been deemed critical for maintaining military health readiness during a pandemic or other public health emergency. As such, the contractor shall provide a Mission Essential Contractor Services Plan to the Government within 30 days of contract award, which addresses the following:

- 5.3.17.1 Challenges associated with continuing performance in a crisis environment,
- 5.3.17.2 The identification of essential personnel and resources required to implement this plan,
- 5.3.17.3 Procedures for recalling personnel who may be on leave or in a non-duty status,
- 5.3.17.4 A plan for acquiring the necessary resources to continue operations for a minimum of 180 days, and
- 5.3.17.5 How the contractor will interface with the Government's crisis management structure

5.3.17.6 The plan will be reviewed on an annual basis or in conjunction with Government-led crisis preparedness exercises.

5.4. Special Qualifications:

5.4.1. When using education/certification in conjunction with labor categories, the COR in coordination with the CO must establish a review process of contractor personnel to ensure labor category requirements are met.

5.4.2. Project Manager. The contractor shall designate a full-time Project Manager and alternate(s). The PM (or alternate) shall be responsible for overall contract performance and shall have full authority to act for the contractors on all matters related to contract operations under their purview.

5.4.3. Contractor Representative (CR). The Contractor shall provide a CR to resolve any administrative, technical issues, and/or problem areas the customers experience in the field. The CR shall be available 24 hours 7 days a week via telephone, and be available in person by appointment, upon request by the CO or the SA COR. The CR shall be available to support each Submitting Activity (SA) within their assigned territory, for services that include but are not limited to, troubleshooting, servicing, and introducing new tests, within 72 hours.

5.4.4. Specimen Processing / Data Entry Technician(s). The Contractor shall provide a technician(s) to process (spin, aliquot), label, and package specimens in the proper shipping envelope and/or container for pickup and delivery to the Contractor's laboratory testing site(s) using proper protective equipment and universal precautions. The technician(s) will ensure the specimens are checked and logged before leaving the SA laboratory shipping room as part of their certification of receipt procedures.

5.4.4.1. The technician(s) shall be responsible for transmitting laboratory test requests, retrieving laboratory results, obtaining test information, printing patient reports, and printing utilization reports from the Contractor's LIS.

5.4.4.2. Technician(s) shall also maintain and order all necessary supplies to support this effort.

5.4.5. Technician(s) will assist with processing specimens and accurately entering laboratory results into MHS Genesis when the interface between the Contractor's LIS and MHS Genesis is not available.

5.4.6. Technicians must comply with required Common (Individual) mandatory training topics IAW DHA Procedural Instructions and the medical treatment facilities' policy. This includes but is not limited to OPSEC, Sexual Harassment/Assault Response and Prevention (SHARP), Anti-Terrorism Level I, Threat Awareness & Response Program (TARP), Information Security, IA, Cybersecurity, and Combatting and Trafficking in Persons.

5.4.7. Customer Services. The Contractor shall provide customer service/support personnel who shall be accessible by a tollfree telephone service 24 hours per day, 7 days per week to assist Government staff in tracking test results, modifying test requests, obtaining information regarding specimen requirements, requesting emergency courier service, using the provided computer system, and other applicable related issues/problems that arise in the performance of work under this contract.

5.4.8. Courier Services. The Contractor shall provide courier service personnel (transportation) to pick up, ship, and deliver specimens from the SA site to the Contractor laboratory.

5.4.9. Couriers will need to access military installations. Access instructions are dependent upon each installation. All costs associated with obtaining access to military installations are the responsibility of the contractor.

5.4.10. The Contractor shall provide routine local courier pick-up services at each SA daily (Monday – Friday,

excluding federal holidays) at a time mutually agreed upon by the SA COR and the Contractor.

5.4.11. Computer / IT Support Personnel. The Contractor shall provide computer support personnel to address MHS Genesis Test File support. Computer support personnel will assist sites with the modification and /or build of Lab Test Files (LTF) to support the vendor's electronic interfacing capabilities to MHS Genesis.

5.4.12. Computer Support Personnel must complete DoD OPSEC, AT Level I, and Cyber Awareness training, at a minimum, and must successfully complete a security investigation, e.g., NACLC or equivalent. The security investigation is the responsibility of the contractor. The Computer Support personnel will need to complete the above tasks to obtain a CAC.

5.4.13. Computer support personnel must meet Automated Data Processing / IT-II for access to MHS systems including MHS Genesis.

5.4.14. When a test is requested that is not already interfaced, the Contractor's Computer Support Personnel shall provide all necessary technical data (e.g., LOINC codes, test parameters, result formats) to the MHS Genesis Test File support team within 2 business days of the request to enable the government team to build and interface the new test within the 7-day requirement..

PART 6

6.0 INFORMATION TECHNOLOGY & SECURITY

6.1 All work under this contract is unclassified.

6.2 The TIER 1 or TIER 2 levels and position sensitivity designation for positions under this contract is:
(Requirement must be checked in order to be a requirement for this PWS.)

6.2.1 TIER II: Non-sensitive position

6.2.2 **Personally Identifiable Information (PII)/Protected Health Information (PHI), Procurement, and Federal information requirements:** Data Sharing Agreements: Contractors requiring access to PII, which includes PHI, or access to de-identified data, are subject to the DHA Privacy and Civil Liberties Office (DPCLO) (Privacy Office) Data Sharing Program. This program requires DHA to enter into DSAs with parties outside the MHS who use or create MHS data. A DHA contract uses the term Data Use Agreement (DUA) rather than DSA. DSAs assure that outside parties protect MHS data IAW the Privacy Act and the HIPAA Rules. To apply for a DSA, the contractor submits a Data Sharing Agreement Application to the DHA DPCLO. The contractor submits the DSAA even if a subcontractor will be the party accessing MHS data. After review and approval of the DSAA, the Privacy Office provides a DSA to the contractor for execution.

6.2.3 Processing Procurement Sensitive Information: All individuals shall seek guidance from the CO regarding the coordination of documents, dissemination, and transmission of procurement SI. Procurement SI shall not be transmitted electronically unless encryption is utilized. Depending on a particular procurement, other restrictions may apply.

6.2.4 Contractor employees, including subcontractors, performing services under this contract with access to PII and Government information shall complete "Identifying and Safeguarding PII" within 30 calendar days of employment. Contractor personnel shall complete refresher training every twelve (12) months from initial completion. The Contractor shall provide the COR a copy of the training certificates for its employees no later than five (5) days after completion. Training is available at <https://jkodirect.jten.mil/Atlas2/page/desktop/DesktopHome.jsf>

6.3 Training

6.3.1 Contractor employees performing cybersecurity/cyberspace functions shall comply with the following requirements:

6.3.1.1 Training: All contractor and associated subcontractor employees working Cybersecurity Information Assurance (IA)/Cyberspace functions must comply with DoD training requirements in Department of Defense Directive (DoDD) 8140.01 and DoD 8570.01-M. Contractors shall identify, document, track, and report qualifications of contract support personnel who perform cyberspace work roles.

6.3.1.1.1 Technician(s) must complete DoD IA, Operations Security (OPSEC) General User Training and successfully complete a security investigation, e.g., National Agency Check with Law Enforcement and Credit (NACLC), or equivalent within 30 calendar days after task order award. The technician(s) will need to complete the above tasks in order to obtain a CAC. Security investigation is the responsibility of the contractor.

6.3.1.1.2 Upon request by the Government, the contractor shall provide documentation supporting the IA certification status of personnel performing IA functions.

6.3.1.2 Contractor personnel who do not have proper and current certifications shall be denied access to DoD IS for the purpose of performing IA functions.

6.3.2 User requirements: All contractor employees that require access to DHA IT must comply with the requirements of DHA-Procedural Instruction 8140.01, Acceptable Use of DHA IT, to include those contract employees with privileged access.

6.4 Cybersecurity Requirements for Non-DoD IT or Covered Contractor IS: .

6.4.1 The contractor shall, at time of award, have implemented the security requirements prescribed in the NIST Special Publication 800-171, "Protecting Controlled Unclassified Information in Nonfederal Information Systems and Organizations" (available via the internet at <http://dx.doi.org/10.6028/NIST.SP.800-171>).

6.4.1.1. The Contractor shall submit a completed System Security Plan (SSP) with any applicable Plans of Action to affirm compliance to implementing the NIST SP 800-171 security requirements. See Contract Data Requirements List 6.4.1P170, System Security Plan and Associated Plans of Action for a Contractor's Internal Unclassified Information System. (PWS Attachment 2)

6.4.2 The contractor shall, at time of award, have posted a current CMMC Final Level 2 (Self) in the Supplier Performance Risk System (SPRS) via the PIEE, (available via the internet at <https://www.sprs.csd.disa.mil/>).

6.4.3 Cyber Incident Reporting Requirement

6.4.4 In the event of a cybersecurity incident involving CUI (i.e., PII/PHI), the contractor shall follow the incident reporting guidance prescribed in the TRICARE Operations Manual, Chapter 1, Section 5, "Compliance with Federal Statutes" at <https://manuals.health.mil/>. In addition, the contractor shall rapidly report (within 72 hours) cyber incidents involving DoD CUI (PII/PHI) to DoD Cyber Crime Center via <https://dibnet.dod.mil/portal/intranet/> IAW DFARS clause 252.204-7012.

6.4.5 The contractor shall maintain within the US or US territories all Government data that is not physically located on DoD premises, unless the contractor receives written notification from the CO to use another location, IAW DFARS 239.7602-2(a).

6.4.6 The contractor shall mitigate supply chain risk to the government by complying with DFARS 252.239-7018, and only utilizing unified capability equipment identified on the DODIN Unified Capabilities Approved Products List (<https://aplits.disa.mil/processAPList>), unless granted a waiver IAW DODI 8100.04, DOD Unified Capabilities (UC).

6.5 Facility Related Control Systems: RESERVED

6.5.1 FRCS Cybersecurity Requirements: RESERVED

6.5.2 Post Award Requirements: RESERVED

6.5.3 Risk Assessment Timeframes: RESERVED

6.5.4 Warranty and Post-Warranty Service Maintenance Agreement: RESERVED

6.5.5 Cybersecurity Requirements/Continuous Monitoring/Risk Management: RESERVED

6.6 Reserved.

6.7 System Authorization Access Request (SAAR), Defense Department (DD) Form 2875

6.7.1 The contractor shall submit the most current version of DD Form 2875, "System Authorization Access Request (SAAR), (PWS Attachment 7), IAW CO guidance for all contractors that use the DoD Gateways to access Government IT systems and/or DoD applications shall. A DD Form 2875 shall be completed for each contractor employee who will access any system and/or application on a DoD network. The DD Form 2875 shall clearly specify the system and/or application name and justification for access to that system and/or application.

6.7.2 The contractor shall submit the completed DD Form 2875 to the DHA IT Security for verification of Tier I or Tier II Designation. The DHA Personnel Security will verify that the contractor employee has the appropriate background investigation completed or a request for background investigation has been submitted to the Office of Personnel Management (OPM). Acknowledgment from OPM that the request for a background investigation has been received and that an investigation has been scheduled will be verified by the DHA Personnel Security prior to access being approved.

6.7.3 Upon approval, DHA will notify the user of the ID and password via secure/encrypted e-mail upon the establishment of a user account. User accounts will be established for individual use and may not be shared by multiple users or for system generated access to any DoD application. Misuse of user accounts by individuals or contractor entities will result in termination of system access for the individual user account.

6.7.4 The contractor shall conduct a monthly review of all contractor employees who have been granted access to DoD IS/networks to verify that continued access is required.

6.7.5 The contractor shall provide the DHA DPCLO with a report of the findings of their review by the 10th day of each month following the review. Reports identifying changes to contractor employee access requirements shall include the name, DoD ID number from CAC, Company, IS/network for which access is no longer required and the date access will be terminated.

6.8 MHS Systems Telecommunications: RESERVED

6.9 Establishment of Telecommunications: RESERVED

6.9.1 See Contract Data Requirements List R230, B2B Gateway Questionnaire (PWS Attachment 3).

6.10 Contractors Located on Military Installations:

6.10.1 The contractor(s) shall coordinate/obtain the connections with the local MTFs/ Enhanced Multi-Service Markets (eMSMs) and Base/Post/Camp communication personnel located on a military installation who require direct access to Government systems.

6.10.2 These connections will be furnished by the Government.

6.10.3 Contractors located on military installations that require direct connections to their networks shall provide an isolated IT infrastructure. They shall coordinate with the Base/Post/Camp communications personnel and the MTF/eMSM in order to get approval for a contractor procured circuit prior to installation to ensure the contractor is within compliance with the respective organizational security policies, guidance and protocols.

6.10.4 The contractor shall be responsible for all security certification documentation to support DoD IA

requirements for network interconnections.

6.10.5 The contractor shall provide, on request, detailed network configuration diagrams to support IA accreditation requirements.

6.10.6 The contractor shall comply with IA accreditation requirements. All network traffic shall be via Transmission Control Protocol/Internet Protocol using ports and protocols IAW current Service security policy. All traffic that traverses MHS and/or military Service Base/Post/Camp security infrastructure is subject to monitoring by security staff using Intrusion Detection Systems.

6.11 **Cyber Security Model Maturity Certification (CMMC) Level Requirement:** The Contractor shall have a minimum of a current (i.e., not greater than three years) CMMC Final Level 2 (Self) score posted in the Supplier Performance Risk System (SPRS) at the time of award. The Contractor shall obtain a current (i.e. not greater than three years) CMMC Level 2 certification issued by an authorized CMMC third-party organization (C3PAO) by November 1, 2028.

PART 7
PWS ATTACHMENTS LISTING

PWS Attachment 1 - DoD Summary of Facilities

PWS Attachment 2 – DD Form 1423-1 -System Security Plan and Associated Plans of Action for a Contractor's Internal Unclassified Information System

PWS Attachment 3 – B2B Gateway Implementation Plan and Questionnaire

PWS Attachment 4 – DHA CAC Request Process

PWS Attachment 5 – DHA Form 33 - Request for Common Access Card (CAC)

PWS Attachment 6 – Example of DHA Common Training Requirements

PWS Attachment 7 – DD Form 2875_System Authorization Access Request

PWS Attachment 8 – Performance Requirement Summary (PRS)

PWS Attachment 9 – Deliverable Schedule